

|   |                                                                                 |                       |
|---|---------------------------------------------------------------------------------|-----------------------|
| 3 | Peripheral neuropathy and a history of foot ulcer or lower-extremity amputation | Once every 1-3 months |
|---|---------------------------------------------------------------------------------|-----------------------|

source : *The IWGDF guidelines 2015*

4.1.2 In a person with diabetes who has peripheral neuropathy, screen for: a history of foot ulceration or lower-extremity amputation; peripheral artery disease; foot deformity; pre-ulcerative signs on the foot; poor foot hygiene; and ill-fitting or inadequate footwear. (Strong; Low) IWGDF 2015 (Adopted)

4.1.3 Treat any pre-ulcerative sign on the foot of a patient with diabetes. This includes: removing callus; protecting blisters and draining when necessary; treating ingrown or thickened toe nails; treating haemorrhage when necessary; and prescribing antifungal treatment for fungal infections. (Strong; Low) IWGDF 2015 (Adopted)

4.1.4 To protect their feet, instruct an at-risk patient with diabetes not to walk barefoot, in socks, or in thin-soled standard slippers, whether at home or when outside. (Strong; Low) IWGDF 2015 (Adopted)

4.1.5 Instruct an at-risk patient with diabetes to: daily inspect their feet and the inside of their shoes; daily wash their feet (with careful drying particularly between the toes); avoid using chemical agents or plasters to remove callus or corns; Adopted use emollients to lubricate dry skin; and cut toe nails straight across. (Weak; Low) IWGDF 2015 (Adopted)

4.1.6 Instruct an at-risk patient with diabetes to wear properly fitting footwear to prevent a first foot ulcer, either plantar or non-plantar, or a recurrent non-plantar foot ulcer. When a foot deformity or a pre-ulcerative sign is present, consider prescribing therapeutic shoes, custom-made insoles, or toe orthosis. (Strong; Low) IWGDF 2015 (Adopted)

4.1.7 Instruct a high-risk patient with diabetes to monitor foot skin temperature at home to prevent a first or recurrent plantar foot ulcer. This aims at identifying the early signs of inflammation, followed by action taken by the patient and care provider to resolve the cause of inflammation. (Weak; Moderate) IWGDF 2015 (Adopted)

4.1.8 To prevent a first foot ulcer in an at-risk patient with diabetes, provide education aimed at improving foot care knowledge and behaviour, as well as encouraging the patient to adhere to this foot care advice. (Weak; Low) IWGDF 2015 (Adopted)

4.1.9 To prevent a recurrent plantar foot ulcer in an at-risk patient with diabetes, prescribe therapeutic footwear that has a demonstrated plantar pressure relieving effect during walking and encourage the patient to wear this footwear. (IWGDF 2015 (Adapted)

4.1.10 To prevent a recurrent foot ulcer in an at-risk patient with diabetes, provide integrated foot care, which includes professional foot treatment, adequate footwear and education. This should be repeated or re-evaluated once every one to three months as necessary. (Strong; Low) IWGDF 2015 (Adopted)

#### 4.2.1: Assessing the risk of developing a diabetic foot problem

4.2.1a Evaluate a diabetic patient presenting with a foot wound at 3 levels: the patient as a whole, the affected foot or limb, and the infected wound. IDSA 2012 guidelines (Adapted)

4.2.1b Assess the affected limb and foot for arterial ischemia (strong, moderate), venous insufficiency, presence of protective sensation, and biomechanical problems. IDSA 2012 guidelines (Adapted)

Explanatory note: Biomechanical problems means anatomical and physiological disturbances of the foot. i.e., structural changes which happen in the bones, joints and muscles of the foot of diabetics and the changes in the blood circulation and nerve sensation of the foot of diabetics.

#### **4.2.2. Classification of Diabetic foot**

**4.2.2a** Assess the severity of any diabetic foot infection using the Infectious Diseases Society of America/ International Working Group on the Diabetic Foot Classification system. (Strong; Moderate) IWGDF 2015 (adopted)

*(Refer to Annexure below for IDSA/IWGDF classification system -Table 1)*

**4.2.2b** Do not use the Wagner classification system to assess the severity of a diabetic foot ulcer. (Adopted) (NICE 2015 Guidelines on Diabetic foot problems: prevention and management.)

#### **4.2.3 Referral for Diabetic foot problems**

**4.2.3a** Initially hospitalize all patients with a severe infection, selected patients with a moderate infection with complicating features (eg, severe peripheral arterial disease [PAD] or lack of home support), and any patient unable to comply with the required outpatient treatment regimen for psychological or social reasons. Also hospitalize patients who do not meet any of these criteria, but are failing to improve with outpatient therapy. IDSA 2012 guidelines (Adapted)

*(Also refer to Table-3 and 4 in the Annexure below, for explanatory notes.)*

**4.2.3b** Prior to being discharged, make sure that a patient with a DFI (Diabetic Foot Infection) is clinically stable; has had any urgently needed surgery performed; has achieved acceptable glycemic control; is able to manage (on his/her own or with help) at the designated discharge location; and has a well defined plan that includes an appropriate antibiotic regimen to which he/she will adhere, an off-loading scheme (if needed), specific wound care instructions, and appropriate outpatient follow-up. IDSA 2012 guidelines (Adapted)

### **4.3: DIABETIC FOOT INFECTIONS**

4.3.1 Consider the possibility of infection occurring in any foot wound in a patient with diabetes. Evidence of infection generally includes classic signs of inflammation (redness, warmth, swelling, tenderness, or pain) or purulent secretions, but may also include additional or secondary signs (e.g., non-purulent secretions, friable or discoloured granulation tissue, undermining of wound edges, foul odour). IDSA 2012 Guidelines (Adapted)

4.3.2 Be aware of factors that increase the risk for diabetic foot infections (DFI) and especially consider infection when these factors are present; these include a wound for which the probe-to-bone (PTB) test is positive; an ulceration present for >30 days; a history of recurrent foot ulcers; a traumatic foot wound; the presence of peripheral vascular disease in the affected limb; a previous lower extremity amputation; loss of